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India was preparing to go under a prolonged lockdown. All train services were suspended at least until 31 March, including suburban rail services and metro services. Goods trains were exempted from this. All interstate passenger transport was also suspended. Domestic flights were to be suspended with effect from the midnight of 24 March.

Meanwhile, it was no joke for the NTF to decipher between what would and would not work in India's context from constantly changing emerging experiences from other countries. Due to the very limited information that the world and the scientific community at large had about the new virus, the questions that they needed to find answers to were how would due processes and SOPs be decided? For instance, as of last week, the UK had withdrawn items of critical advice which had previously stated that asymptomatic patients could not spread the infection. A few days later, their National Health Service instructed hospital trusts to urgently discharge all medically fit patients as soon as it was clinically safe, ideally within three hours. The directive essentially was to get people out of the hospitals to their homes as soon as possible with the right support. They were calling this model 'discharge to assess (D2A)'. This model did not require testing patients for COVID-19 and waiting for them to test negative in order to discharge them. How could we be sure that we are not releasing asymptomatic carriers of the infection back into the community? The challenge was for how long could we afford to let patients occupy hospital beds if they appeared clinically healthy even though they were not testing negative on the COVID-19 test versus freeing up hospital beds faster? In such a scenario, would we be risking releasing potential spreaders into the community?

Since the ICMR constituted the NTF, it was important for them to take a comprehensive look at how most countries were competing for the same products—PPE, masks, sanitizers and ventilators—essentially coming out of Asia. The US, in fact, announced that all states must make their own arrangements to stockpile these products, essentially pitting one state against the other. While India was manufacturing some of these, we didn't have nearly enough of the essentials. To compete with other wealthier economies, the NTF and subsequently the Empowered Group No. 6 mandated to ensure availability of medical supplies, would have to figure